

DOCUMENT	RS-PITCH-01
REVISION	0.1 — DRAFT
SHEETS	20

ISO 13485 · EU MDR · IVDR

Regulatory Sidekick

Regulatory consultants sell fear. We sell the map. **A stepwise implementation of your QMS and technical documentation**, one activity at a time — so compliance stops being something that happens to you and starts being something you did.

Two engineers, four years, one working device.

They have a prototype that works. They have clinicians who want it. They have a seed round that assumed a CE mark by Q3, and a burn rate that makes that assumption expensive.

What they do **not** have is anyone who has done this before. No QA hire. No RA hire. Between them: **zero days** of experience building a quality management system — and a shared, entirely reasonable belief that it can't be that hard to write some procedures.

“Then a notified body sends a list of what it will need to see, the founders read it twice, **and it may as well be in another language.**”

An archetype, not a customer — deliberately. We have no named references yet (see §3.6). Every detail is drawn from the segment profile, not from a real company we are quietly borrowing.

It isn't that the rules are secret. It's that nobody will tell them the order.

01

Everything looks like step one

Risk management, design controls, clinical evaluation, supplier qualification, post-market surveillance — every guide insists its own topic comes first. **None of them agree**, and none mentions what has to exist before you can start.

02

“Enough” is never defined

Is a three-page procedure acceptable, or negligent? The standard does not say. So the safe answer is always **more** — more pages, more process, more of the founder's remaining year.

03

The clock is the real antagonist

Every month spent guessing is a month of burn against a milestone the round was priced on. They cannot afford to be slow and they cannot afford to be wrong. **So they freeze.**

The blocking question is never “**what does the standard say?**” — that text is free and always has been. It is “**what do I do first?**”

ANSWER ONE — “DON’T TOUCH IT”

Buy the expertise by the day

€800–1,500 a day, 40–80 days. It works — and it costs more than the round can spare, bills setup and templates at the same rate as the judgment, and when the invoice is paid **the understanding walks out with it.**

Next year, when the device changes, they call again. And the year after that.

€35k–100k

PER IMPLEMENTATION

ANSWER TWO — “HERE, GOOD LUCK”

Buy a folder of templates

200 documents and no idea which to open on Monday. Templates without a path are shelfware — and a procedure you did not reason through is one you **cannot defend when an auditor asks why.**

The folder is opened once, then never again, and the deadline arrives anyway.

€1.5k–5k

PER TEMPLATE PACK

Both answers quietly agree on the same premise — **that this is not something the founders can learn.** One sells them dependence, the other abandonment. Nobody in the room is selling them **competence**, because competence, unlike hours, only sells once.

ACT TWO

The turn

What if the premise is simply wrong? The Medical Device Regulation is a long, dry, entirely knowable document. People learn far harder things than this on purpose. The barrier was never the difficulty — **it was that nobody had written down the order.**

Compliance is sold as danger. We teach it as craft.

WHAT THE FOUNDERS WERE TOLD

It is too complex to learn. Acronyms left unexplained, because unclear is what is billable.

You are already behind. The audit date used to compress decisions they have not had time to understand.

You cannot do this yourself. Stated as a fact about the regulation. It is a fact about the business model.

All of it might apply to you. Nobody is paid to tell a client that a requirement is irrelevant to their device.

WHAT IS ACTUALLY TRUE

Every requirement has an address. ISO 13485 §7.3.2, MDR Annex I 23.4 — cited, checkable, and if we cannot cite it we say so.

There is a defensible order. Real dependencies, so you can see what is next, what is blocked, what can wait a quarter.

You can — starting this week. Each of the 116 activities has a lean version small enough to actually begin.

Most of it does not apply. The device profile switches off what is not yours.
Removing work is the feature.

This is not anti-consultant. The good ones are worth their rate, and our founders will still want one for the genuinely hard calls — classification, a novel clinical strategy, the week before an audit. What we object to is being *kept* in the dark as a business model. A founder who can read their own clause map is a better client, not a lost one: they stop paying senior people to write boilerplate and start paying them for judgment.

What they needed was never a saviour. It was a sequence.

I — THE MOAT

Sequencing

116 activities in a defensible order across four phases, with real dependencies encoded. Templates are everywhere; **an order of operations is not**. This is the scarce thing, and it is what we are actually selling.

II — THE SUBSTANCE

Artefacts

275 controlled-document templates — procedures, work instructions, forms and registers — already interlinked, clause-mapped, and each traced to the process step that produces it.

III — THE PROOF

Traceability

A clause-level matrix across 14 standards plus a per-activity evidence trail — what you actually slide across the table when an auditor asks you to show your work.

Why the name is the strategy. A sidekick never takes the hero’s place — that is the entire job description. We are not an eQMS (they sell the container and leave it empty, which makes them partners rather than competitors), and we are not a consultancy: we sell the knowledge itself, once, instead of the hours of applying it — which is precisely why **the incentive to keep it mysterious never appears**.

Four phases, and permission to start small

PHASE I

Set the foundations

Stand up the lean QMS and define the product — intended purpose, qualification, classification.

PHASE II

Develop under control

The design & development heavy-lift: plans, software, and the initial risk and clinical analyses.

PHASE III

Verify, validate & make

Execute V&V, complete clinical, performance and usability evidence, stand up production.

PHASE IV

Certify, launch & operate

Compile the technical file, pass the notified-body assessment, then run PMS and vigilance.

First, we make the plan smaller

A one-time device profile filters everything on two axes: **regulatory route** (MDR or IVDR — mutually exclusive) and **11 device characteristics** (software, AI/ML, IVD, self-test, companion dx, hardware, active, sterile, connected, personal data, US FDA). Our two founders never open a page about sterilisation they do not need.

Then we make step one small

Every activity carries a **minimum-viable version** they can do this week and an **evolve path** for when they are bigger. A three-page procedure a company follows beats a forty-page one it does not — an auditor checks whether you do what you say, not whether you wrote a lot.

Six months later, the same two people

BEFORE

“We think we need a risk file? Someone said we need a risk file.”

A quote for 60 consultant days they cannot fund without cutting an engineer.

Documents someone else wrote, in a folder, unread.

“How’s the QMS going?” answered with a paragraph and a wince.

AFTER

“Risk management is **done** — ISO 14971, the analysis and the report, signed by a named person on a date.”

A consultant booked for **four days**, on the two questions that genuinely need one.

A quality system **they can explain out loud** — because they built it, in order, with the clause next to each step.

“Phase III, 68% complete, two blocked on supplier data.” **An answer with a number in it.**

This is the product’s real promise — not documents, but a team that understands its own quality system well enough to defend it, change it, and carry it into the next device without starting over.

None of that works unless the corpus is real

Sequenced activities	116	Controlled documents	275
Deep sub-activities	338	Mapped QMS processes	39
Granular checklist tasks	682	Standards, clause-level	14
Implementation phases	4	Quality System Essentials	12

STANDARDS COVERED CLAUSE BY CLAUSE

ISO 13485 · EU MDR · EU IVDR · ISO 14971 · IEC 62304 · IEC 62366-1 · IEC 81001-5-1 · IEC/TR 80002-2 · AAMI TIR57 · MDCG guidance · ISO 27001 · EU AI Act · GDPR · FDA QMSR

Enforced, not asserted: the build fails if a single document has no producing process step, if a reference is broken, or if a step consumes a document produced in a later phase. Every figure above is counted from the shipped content at build time, and the traceability claim is checked by CI on every deploy.

The first QMS implementation an AI agent can actually work

Sold as a separate entitlement on top of the licence — not bundled, not free.

I

It reads the same map

An agent asks the API what is startable now and gets the identical ordered list the founders see on their dashboard — with the why, the lean version, the clause map and the documents each step produces.

II

Governed like a record

Keys are member-created, **admin-approved**, scope-limited, expiring, revocable and rate-limited. Every write lands in the audit log attributed to a named human, tagged *via agent*.

III

It never signs off

It drafts; it may set *In progress*; it can never mark an activity **Done** or invent a date, signature or test result. Closing an activity stays a named person taking responsibility.

This is the answer to “won’t an LLM just do this?” — **the model supplies fluency; we supply the sequence, the clause map and the governed record**. An agent with our API drafts a QMS. An agent without one produces plausible documents nobody can audit, which is the old problem wearing a new coat.

ACT THREE

The business

Our two founders are one company. There are thousands of them, plus the consultancies who serve them and the funds who bankroll them — and all three are buying the same missing thing. **Here is what that is worth.**

Who else walks this road

SEGMENT	WHO THEY ARE	PRICE POINT	WHY THEY BUY
A • Medtech & IVD startups	Our two founders. 2–15 people, first device, no QA/RA hire	€1,800	Investor diligence, first clinical study, CE-mark deadline
B • SME manufacturers	15–200 people, MDD-to-MDR transition or scaling a QMS	€6,000	Notified-body audit date, MDR transition deadline
C • QA/RA consultancies	Deliver builds to 3–10 clients a year; stop rebuilding the same scaffolding each time	€3,000–8,000 / yr	Every new engagement — recurring by construction
D • Medtech funds	20–40 portfolio companies continuously, platform budget, capital exposed to delay	€12,000–15,000 / yr	One block replaces ten sales — renews with portfolio refresh

Why C and D matter disproportionately: a one-time product has no revenue floor — every euro must be re-sold next month. The consultancy and fund licences are the only motions that both reach many end users per transaction **and** renew annually, which is what fixes the non-compounding revenue problem.

A ladder, not a single price

The failure mode of a knowledge product is one price with a cliff in front of it. Each rung does a distinct job.

RUNG	PRODUCT	PRICE	JOB IT DOES
0	Explore	Free	Browse the whole roadmap, matrix and library index; one sample activity open in full — proves depth before commitment
1	Readiness self-assessment	Free, email-gated	Top of funnel, and a qualification signal
2	“Stage-1 Ready” pack	€690	The missing rung — the 12 core processes needed to survive a Stage-1 audit; credit-back toward full access
3	Practitioner	€1,800 • €600×3 / €300×6	Segment A — self-declared eligibility, up to 3 users, single device family
4	Standard	€6,000 • €2,000×3 / €1,000×6	Segment B — companies and teams rolling out their own QMS
5	Content-currency renewal	20–25% of purchase / yr	The only honest recurring layer — earned by a quarterly “what changed, and which of your documents it touches” briefing
6	Consultancy & fund licences	€3,000–15,000 / yr	Multi-client delivery rights and portfolio blocks — recurring from day one

~91% margin, payback on day zero

PER BLENDED SALE	AMOUNT
Blended ASP (70/30)	€3,060
Support (~2h) + hosting	-€180
Payment processing (~3%)	-€92
Gross profit	€2,788

One-time, collected upfront, so **payback is immediate** — customers can be bought profitably from day one. At an €800 CAC that is 3.5:1 on the first purchase alone. The constraint is therefore **distribution, not product**.

YEAR ONE	CONSERVATIVE	BASE	UPSIDE
Stage-1 packs · €690	20 · €13,800	60 · €41,400	150 · €103,500
Practitioner · €1,800	12 · €21,600	30 · €54,000	60 · €108,000
Standard · €6,000	4 · €24,000	12 · €72,000	28 · €168,000
Consultancy licences	1 · €3,000	4 · €12,000	10 · €30,000
Fund blocks	—	1 · €12,000	3 · €45,000
Revenue	€62,400	€191,400	€454,500
Deals / month	~3	~9	~21

Read this honestly. Modelled scenarios, not forecasts from observed conversion — there is no revenue history behind them yet. Fund blocks are excluded from the conservative case on purpose: that motion cannot start until reference customers and the investor view exist. The base case is **nine transactions a month** — a marketing problem, not a product problem.

How we find the next thousand of them

01

Consultancies as a channel

They already carry the client relationship we would otherwise pay ~€800 CAC to acquire. The licence covers **delivery, not resale**, and when an engagement ends the client workspace converts to a direct licence at 50% credit — a funnel, not a leak. It is also the proof that our position is **demystify, not displace**: the practitioners who adopt us already work this way.

02

Medtech funds

A fund holds 20–40 portfolio companies continuously, has a platform budget, and has its own capital exposed to regulatory delay. One €12–15k block replaces ten individual sales — and **changes who the buyer is**, from founder runway to platform budget. The argument that lands is **legibility, not cost**: burn and hiring are instrumented, and then “how’s the QMS going?” returns a paragraph.

03

A named practitioner, publishing

In regulatory affairs trust is personal. A credentialed practitioner writing consistently outperforms any brand account — and an anonymous brand cannot sell this at all.

04

Accelerators & long-tail search

Cohort deals with medtech accelerators and university TTOs, plus checklist and comparison search terms — where intent is unusually high and the incumbents are content farms.

Why this holds once someone notices it is working

The asset is the content, not the code

If the codebase were lost it would be rebuilt in about six weeks. If the corpus were lost there is no business. Years of practitioner judgment — sequencing, dependencies, clause maps — is the part that cannot be scraped or prompted into existence.

Regulatory content decays, legitimately

MDCG guidance is revised, harmonised standards get new editions, IVDR dates move. A QMS built on 2026 guidance is stale by 2028. That is a **genuine** recurring need, which is what lets a one-time product carry an honest subscription.

The switching cost is their own history

Progress, evidence and an immutable activity log live per workspace. A regulated manufacturer needs **its own** auditable trail — precisely what it can never get from a shared folder of leaked templates.

And we never hold their work hostage

A lapsed licence stops the updates; it never revokes access to documents they already own. Turning a billing lapse into an audit incident would betray the entire premise — **and it is written into the code, not the marketing.**

Where the story actually stands today

BUILT AND RUNNING

- The full corpus — **116 activities, 275 documents**, clause matrix, process map
- Multi-tenant app with row-level security, roles and an immutable audit log
- Device-profile scoping, evidence uploads, per-workspace progress
- Self-serve checkout with instalments, EU VAT and reverse charge
- Agent API v1 with admin approval, scopes and rate limits

NOT YET — AND NEXT, IN ORDER

- Free **readiness self-assessment** — top of funnel
- **€690 Stage-1 pack** — removes the free-to-€1,800 cliff
- Read-only **investor view** — prerequisite for the fund motion
- First **quarterly regulatory changelog** — the renewal product
- Two to three **named reference customers** and one case study

Traction — paying customers to date, revenue booked, design partners in conversation — left blank rather than estimated. The plan is to sell ten accounts by hand before spending anything on acquisition; if ten cannot be closed from warm outreach, the problem is positioning, not price. **The founders in §1.1 are an archetype for exactly this reason: we have not yet earned the right to put a real name there.**

PRE-REVENUE

What could end it

RISK	SEVERITY	HOW IT IS HANDLED
Regulatory liability	High	We supply documents used in conformity assessment. Explicit disclaimer on every template and in the licence — a starting point, not regulatory advice; the manufacturer remains solely responsible . Professional indemnity cover from day one, counsel review before public launch. Not a corner to cut.
Leakage	High	275 downloadable files, one buyer shares them. Per-org watermarking and doc-code footers, licence terms — and structurally, we sell the path and the updates , not the files. A regulated buyer needs its own auditable trail, which a shared zip cannot provide.
Content decay	High	Guidance and harmonised standards move. Mitigated by making currency the paid renewal, with real founder time budgeted quarterly — the maintenance <i>is</i> the recurring product.
No compounding revenue	Structural	One-time pricing has no MRR floor; a slow month is a zero month. Addressed by the entry rung, the currency renewal, and the consultancy and fund licences — never “one price, sold once”.

Also live: trust deficit (a named practitioner must front the brand), refund exposure, and cross-border EU VAT/OSS — the last handled in checkout from the first sale rather than retroactively.

Sell ten by hand. Watch whether they finish.

In a knowledge product, **completion is retention** — and the only honest proof that understanding transferred, rather than another document set changing hands. So the first milestone is not revenue. It is **two founders who now know what they are doing**, and who will say so out loud to the next two.

10

DESIGN PARTNERS

60%

ACTIVATION IN 7 DAYS

40%

25%+ DONE AT DAY 30

The ask

AMOUNT · USE · TIMELINE

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Coral-marked fields need your input before this deck leaves the building. The founders in §1.1 are an archetype, not a customer. Financial figures in §1.3, §3.1 and §3.3 are internal modelling assumptions, not researched market data or observed results.